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Tactical Doctrine

GROUND SURGICAL TEAM (GST)

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The Air Force Tactics, Techniques, and Procedures (AFTTP) 3-42 series of publications is the primary reference for expeditionary medical support capabilities. AFTTP 3-42.77 provides the tactics, techniques, and procedures for Ground Surgical Teams (GSTs) tasked to operate independently of the Expeditionary Medical Support (EMEDS) construct. These capabilities include unit type codes (UTCs) FFGST, Ground Surgical Team, and FFGS1, Ground Surgical Equipment. The doctrine in this document is authoritative but not directive. This publication applies to the Regular Air Force, the Air Force Reserve, and the Air National Guard. This publication does not apply to the United States Space Force. Ensure all records generated as a result of processes prescribed in this publication adhere to Air Force Instruction (AFI) 33-322, *Records Management and Information Governance Program,* and are disposed in accordance with the Air Force Records Disposition Schedule, which is located in the Air Force Records Information Management System. Refer recommended changes and questions about this publication to the office of primary responsibility (OPR) using Department of the Air Force (DAF) Form 847, *Recommendation for Change of Product.* Route DAF Form 847 through the appropriate functional chain of command and parent major command. The use of the name or mark of any specific manufacturer, commercial product, commodity, or service in this publication does not imply endorsement by the Department of the Air Force.

SUMMARY OF CHANGES

This revision incorporates changes from recent programmed UTC field development exercises and program reviews. Key changes include refinement of the GST's scope of care and clarification of each team member's primary role and recommended cross-functional support functions. Office

symbols have been updated where appropriate to reflect the organizational changes within the Air Force Medical Service (AFMS). Numerous administrative changes have been implemented throughout. This publication should be completely reviewed.

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Chapter 1

INTRODUCTION

1.1. Purpose. This publication provides the tactics, techniques, and procedures for the Ground Surgical Team (GST). The GST provides the initial surgical and post-surgical critical care capabilities for the Air Force Expeditionary Medical Support (EMEDS) construct. The team can also operate independently in austere locations forward of an established deployed medical facility. This publication focuses on the team's stand-alone and crisis response capabilities. It provides a source document for developing standardized policies, operating procedures, training programs, and allowance standards. Operation plans and regional guidance provide mission-specific information that amplify and tailor the guidance contained in this publication. See AFTTP 3-42.71, *Expeditionary Medical Support (EMEDS) and Air Force Theater Hospital (AFTH),* for more information on EMEDS operations.

1.2. Background. The GST was developed in response to growing combatant command requirements for emergency medical and trauma management support in austere environments. Limited medical infrastructure and long travel distances to the nearest trauma care capability within their areas of responsibility led to an increased demand for small-scale, mobile trauma teams able to deliver life-saving surgical and emergency medical interventions closer to forces. These requests exceeded the available medical support assets in the Air Force Medical Service (AFMS) inventory. Historically, the Mobile Field Surgical Team and a tailored Expeditionary Critical Care Team from the EMEDS construct were often tasked to fulfill these missions. However, these teams were not organized, trained, or equipped to support persistent austere missions. To meet these operational challenges, the Air Force Surgeon General tasked the three medical manpower and equipment force packaging (MEFPAK) responsible agencies to review their unit type code (UTC) portfolios and identify areas where similar training and mission sets could be leveraged. The MEFPAK responsible agencies from Air Combat Command (ACC), Air Mobility Command (AMC), and Air Force Special Operations Command (AFSOC), in partnership with medical consultants and select subject matter experts, identified three ground medical UTCs within the EMEDS construct that could be modified to meet the austere mission requirement without jeopardizing other operational requirements.

1.2.1. The GST replaced EMEDS UTCs: FFMFS, Mobile Field Surgical Team, FFEP1, Expeditionary Critical Care Team, and FFEP5, Surgical Augmentation Team. Positions on the decommissioned UTCs transferred to the GST or to other EMEDS UTCs. A health services administrator position was included to ensure dedicated patient administration, logistics coordination, and medical command center support.

1.2.2. An equipment package was developed specifically for the GST's austere mission. This equipment package is separate and distinct from the EMEDS equipment set. Medical supplies and medications are prepacked to support a rapid and scalable response. The GST equipment allowance standards were reviewed and refined during subsequent field development exercises and program reviews to bolster damage control surgery and resuscitation capabilities and ensure ongoing mission readiness.

1.2.3. A specialized training pipeline was developed for personnel assigned to the GST. Phase 1 focuses on critical medical decision making skills in resource constrained environments, working effectively as a small team, key operational concepts, and equipment familiarization.

Phase 2 provides realistic field operational training tied to the team's mission essential tasks and deployment scenarios.

Chapter 2

GROUND SURGICAL TEAM STAND-ALONE CAPABILITIES

2.1. Mission. The GST provides damage control surgery, damage control resuscitation, and emergency medical care of injured and critically ill patients in dynamic, austere environments outside of established support and patient movement capabilities. The team's primary mission is to stabilize and prepare patients for casualty evacuation (CASEVAC). The team is equipped to perform up to 10 surgical interventions and provide postoperative patient holding for up to 3 patients at a time for a maximum of 12 hours without resupply.

2.2. Scope of Care. The GST performs damage control and emergency medicine interventions required to save life, limb, and eyesight and stabilize critically injured patients for transport. The team's capabilities include multi-system trauma management, blunt and penetrating trauma management, shock and hemorrhage control, respiratory and airway management, initial burn resuscitation and limited ongoing management of thermal injuries, limb revascularization, stabilization of fractures, and major wound debridement. Surgical techniques focus on hemorrhage and contamination control. Weight restrictions and resource constraints limit the amount of crystalloid and blood products available for fluid resuscitations.

2.2.1. The GST can perform limited, type-specific emergency whole blood collection and transfusion in emergent situations in which their existing blood supplies are insufficient.

2.2.2. No formal blood grouping, typing, or crossmatching capability is available. Low-titer O whole blood, uncrossmatched Group O packed red blood cells and Group AB or A fresh frozen plasma or liquid plasma, if available, will be issued for all casualty care.

2.2.3. The GST has no formal laboratory capabilities. Lab testing is limited to point of care testing to assess blood gas, electrolyte, and basic blood count and biomarker data.

2.2.4. Medications are limited to essential emergency medical care, resuscitative surgery, and anesthetic pharmaceuticals.

2.2.5. Radiographic imaging is limited to handheld sonography testing.

2.3. FFGST, Ground Surgical Team. Table 2.1 lists the personnel details for the GST. All team members provide cross-functional support as needed commensurate with their skill levels and training. See **Chapter 3, OPERATIONS,** for more information on deployment and employment.

Table 2.1. FFGST Composition.

AFSC	Title	Grade	Quantity
045S3	Surgeon	04	1
045A3	Anesthesiologist	04	1
041A3	Health Services Administrator	04	1
044E3	Emergency Medicine Physician	04	1
046N3E	Critical Care Nurse	04	1
4N171	Surgical Technologist Craftsman		1
			Total: 6

AFSC	Title	Grade	Quantity
Note: Air Force Specialty Code (AFSC), grade, and skill-level substitutions are in accordance with the *War and Mobilization Plan, Volume 1 (WMP-1), Enclosure F, Medical Supplement*, and the mission capability statement.			

2.4. FFGS1, Ground Surgical Equipment. FFGS1 provides initial supplies and equipment in scalable, portable field packs configured to support 3, 6, or 10 damage control surgeries or resuscitations in austere environments. The equipment set can be tailored for unique operational requirements. See the allowance standard (AS 938Q) for a current list of equipment. Allowance standards are maintained in the Air Force Medical Logistics Allowance Standard Management System (**https://medlog.us.af.mil/areas/afmedlog/#home** [account required]).

Chapter 3

OPERATIONS

3.1. Personnel Deployment Preparations. Team members should work closely with the medical readiness office and theater medical personnel to prepare for deployment. They should review operation plans, reporting instructions, threat and vulnerability assessments, intelligence reports, and other pertinent deployment data. Deploying personnel are responsible for following the pre-deployment requirements outlined in AFI 10-403, *Deployment Planning and Execution.* The UTC team chief should ensure that deploying team members meet the tasking requirements defined by the supported command.

3.2. Equipment Deployment Preparations. The UTC team chief should review the deployability of the GST equipment package with medical logistics personnel to identify and address shortfalls. Unit commanders should ensure an adequate number of personnel are qualified in pallet build-up and cargo handling procedures (to include hazardous, protected, and classified cargo) to serve as cargo increment monitors during deployment and redeployment in accordance with AFI 10-403.

3.2.1. Hazardous cargo is subject to Defense Transportation Regulation (DTR) 4500.9-R-Part III, *Mobility*, Air Force Manual (AFMAN) 24-604, *Preparing Hazardous Materials for Military Air Shipments*, and host nation requirements. The health services administrator and surgical technologist receive training in hazardous cargo packing, handling, and inspection procedures and typically serve as certifying officials.

3.2.2. Protected cargo includes controlled substances, items vulnerable to theft, and weapons and ammunition. A controlled medical item custodian should witness and verify the packaging of medically controlled items. The anesthesiologist typically performs this function. At the deployed location, controlled items should be stored in locked rooms or containers and managed in accordance with Department of the Air Force Instruction (DAFI) 41-230, *Medical Logistics Support,* and local procedures. Units are responsible for assigning primary and alternate weapons and ammunition couriers to ensure security and accountability during transit in accordance with DAFI 31-101, *Base Defense Operations*, and Department of the Air Force Manual (DAFMAN) 21-201, *Munitions Management*.

3.2.3. Classified material should be packaged, marked, safeguarded, and transported in accordance with DAFI 24-602, *Volume 2, Cargo Movement.* Units are responsible for assigning appropriately cleared and trained couriers to accompany classified material.

3.3. Employment. The GST is the smallest deployable conventional Air Force module for delivery of advanced emergency medical and surgical care and is equipped for rapid setup. The team can provide initial stabilization within 15 minutes of arrival at the employment location and reach full mission capability within 30 minutes. The team operates from shelters of opportunity if not attached to an established EMEDS or other medical facility. In austere environments, the team may need to change locations multiple times on short notice. Optimally, the GST should operate from a location with adequate security and base operating support to complete their mission. **Table 3.1** lists the most common employment scenarios.

Table 3.1. Employment Scenarios.

Tasking	Description
Forward deployment	The GST can be tasked to deploy from an EMEDS location to temporarily support operations in austere locations without immediate access to damage control surgery and resuscitation capabilities.
Standalone medical operations	The GST can function as the sole medical resource in a variety of scenarios such as contested environments, humanitarian assistance, disaster relief, and unconventional operations.
Surge support	The GST can deploy to provide surge support for other deployed medical capabilities. The team and equipment set can be used to increase operative and postoperative care capabilities.

3.3.1. Initial operational capability is defined as patient flow areas are established and the team has their equipment in place to perform initial damage control resuscitations.

3.3.2. Full operational capability is defined as the team is ready to perform surgery.

3.4. Command and Control (C2) Structure. In standalone operations, the GST operates under the C2 structure established by the air expeditionary task force or joint task force (JTF) to which they are assigned. Upon arrival in theater, the GST should request a chain of command briefing from the Air Force Forces Surgeon (AFFOR/SG) or JTF/SG office. Usually, the Commander, Air Force Forces (COMAFFOR) retains administrative control. The Service component retains operational control. Tactical control is determined by theater policy. **Note:** When deployed as part of an EMEDS assemblage, GST personnel operate under the local direction of the deployed expeditionary unit commander (usually the EMEDS or AFTH commander).

3.5. Team Member Roles and Cross-Functional Support. Each member of the GST provides a specific skill set and performs a unique function. Team integrity should be maintained to the maximum extent possible. Due to the small size of the team and unique mission, all team members (clinical and non-clinical) should have a basic understanding of each team member's role and be prepared to provide cross-functional support as needed commensurate with their skill levels and training.

3.5.1. The anesthesiologist provides perioperative anesthesia care to surgical patients to include preoperative evaluation and resuscitation, intraoperative anesthetic management, postoperative stabilization, sedation, pain management, and preparation for transport. The anesthesiologist also provides hemodynamic and respiratory support, sedation, and pain management for non-surgical candidates. The anesthesiologist serves as the GST's controlled substances officer and is responsible for maintenance and inventory of the team's medication supply with support from the critical care nurse. The anesthesiologist can assist with preoperative and postoperative clinical care and help maintain the GST's blood supply inventory.

3.5.2. The critical care nurse is responsible for emergency care, perioperative nursing, postoperative critical care, and preparation of patients for transport. The critical care nurse manages and inventories the GST's blood supply and can assist the anesthesiologist in maintaining the team's controlled substance and medication supplies.

3.5.3. The emergency medicine physician, in collaboration with the general surgeon, is responsible for initial resuscitation, triage, pre-hospital care, advanced critical care life support,

and trauma support. Responsibilities include basic management of toxicological emergencies, minor surgical stabilization procedures, limited management of thermal injuries, hemodynamic and respiratory support, sedation, and pain management. The emergency medicine physician can assist the general surgeon in surgery, provide postoperative support, and help prepare patients for transport.

3.5.4. The general surgeon, in collaboration with the emergency medicine physician, is responsible for initial resuscitation, triage, pre-hospital care, advanced critical life support, and trauma support. Responsibilities include evaluation and treatment of traumatic injuries and surgical diseases, performance of minor and major surgical procedures (to include damage control surgery), and critical care of traumatically injured and postoperative patients. The surgeon determines the disposition and prioritization of surgical patients in collaboration with the anesthesiologist and emergency medicine physician.

3.5.5. The health services administrator is the only non-clinical position on the team. The health services administrator's primary responsibilities include coordinating logistics, communications, and CASEVAC support, reporting, and administration functions. As the plans and operations officer, the health services administrator develops and executes logistical, strategic, and tactical plans related to the team's mission capabilities in consultation with the other team members. The health services administrator advises commanders at the tactical and operational levels on the team's capabilities, limitations, and unique planning considerations.

3.5.6. The surgical technologist is responsible for managing the operating room and serves as the general surgeon's primary surgical assistant. Responsibilities include prepping patients for surgery, assisting with patient care interventions, managing surgical instrument sets, and disinfecting surgical instruments. The surgical technologist serves as equipment custodian and assists the health services administrator with ordering medical supplies.

3.6. CASEVAC. The GST operates in areas without close aeromedical evacuation support. The GST health services administrator is responsible for coordinating CASEVAC through the host unit or appropriate theater patient evacuation control center in accordance with theater policy. Patients should be transported with their medical records, personal effects, and medically essential items. The GST is not equipped to furnish the medical equipment and supplies required to support patients during transport. The attending physician is responsible for ensuring that all necessary forms (such as Department of Defense (DD) Form 1380, *Tactical Combat Casualty Care [TCCC] Card,* or Air Force [AF] Form 3899, *Patient Movement Record)* are completed in accordance with DAFI 48-107, *Volume 3, En Route Care Documentation.*

3.7. Documenting Patient Care. Medical personnel document the care provided to U.S. personnel in accordance with Department of Defense Instruction (DODI) 6490.03, *Deployment Health,* AFMAN 41-210, *Patient Administration Support*, and theater surgeon policy. Patient encounter data should be filed electronically through the theater's established electronic health record system (MHS GENESIS). If access to the electronic system is not available, medical personnel can use paper forms. Examples include Standard Form (SF) 600, *Chronological Record of Medical Care*, DD Form 1380, DD Form 3019, *Resuscitation Record,* and AF Form 3899.

3.8. Care for Enemy Prisoners of War. If tasked to treat an enemy prisoner of war, the GST should coordinate with security forces or equivalent authority to provide armed guards for prisoners. Following essential care, enemy prisoners of war are transferred to host nation or U.S. Army prisoner of war management authorities. Care for enemy prisoners of war and medical care

documentation requirements are in accordance with DODI 2310.08, *Medical Program Support for Detainee Operations.*

3.9. Operational Reports. The team chief is responsible for submitting required operational reports in accordance with AFMAN 10-206, *Operational Reporting (OPREP),* and specific combatant commander, joint force commander, and COMAFFOR requirements. Team chiefs submit after-action reports and lessons learned in accordance with AFI 10-204, *Air Force Service Exercise Program and Support to Joint and National Exercise Program,* and AFI 10-1302, *Air Force Lessons Learned Program*.

3.10. Maintenance and Disposition of Records. Records associated with GST operations should be maintained and disposed in accordance with the Air Force Records Disposition Schedule. Patient medical records, personally identifiable information, and protected health information are managed and protected in accordance with AFI 33-332, *Air Force Privacy and Civil Liberties Program,* and AFMAN 41-210. Medical personnel comply with Public Law 93-579 (as amended) Privacy Act of 1974, Public Law 104-191 (as amended) Health Insurance Portability and Accountability Act (HIPAA) of 1996, Freedom of Information Act, Drug Abuse and Treatment Act, and Comprehensive Alcohol Abuse amendments.

3.11. Redeployment. The team can repack its equipment and supplies and be ready to redeploy to a new location within 30 minutes. The health services administrator is responsible for chain of custody and redeployment of the equipment package. Before departure, the team chief should provide the AFFOR/SG with shipment status and a copy of the shipping documents.

Chapter 4

COMMUNICATIONS AND INFORMATION SYSTEMS

4.1. Communications Support Requirements. The GST is not equipped with the communications infrastructure needed for independent operation and relies on host base communications units. The team requires access to the Defense Switched Network, secure voice communications, Non-classified Internet Protocol Router Network (NIPRNET), and SECRET Internet Protocol Router Network (SIPRNET) to communicate patient conditions and patient movement requirements. See **Attachment 2, EXPEDITIONARY COMBAT SUPPORT REQUIREMENTS,** for more information on communications requirements.

4.2. Communications Equipment. The GST deploys with a satellite/iridium phone. These devices are allocated in accordance with the allowance standard and operational requirements. They are primarily used during deployment and redeployment phases and to augment established communications infrastructure in austere environments.

4.3. Computer Systems. The computer issued to the GST is compliant with deployed force information system naming conventions, information assurance requirements, and published theater minimum hardware standards. It includes the DOD Standard Desktop Configuration and applications for managing patient encounter data and ordering medical supplies. Medical applications are provided through the Air Force Operational Medicine Information Systems (AFOMIS) Program Office. Contact the chief information officer, major command, or the ground medical MEFPAK responsible agency for the current list of medical applications included on GST computers.

4.4. Information Assurance Policy. GST personnel should understand and follow information assurance procedures, to include communications and computer security, in accordance with AFI 17-130, *Cybersecurity Program Management,* DAFMAN 17-1301, *Computer Security (COMPUSEC)*, and associated Air Force information assurance guidance.

4.5. Helpdesk Support and System Maintenance. Contact the AFFOR/SG for assistance with computer hardware upgrades and maintenance, software updates, and helpdesk support in the deployed setting.

Chapter 5

INTEGRATION, INTEROPERABILITY, AND DEPENDENCIES

5.1. Integration and Interoperability with Other Systems. The GST can support elements of an air expeditionary force, components of the en route continuum of care system, joint medical counterparts, Special Operations Forces, and other federal and civilian agencies. The GST functions as an integral part of the supported unit and should participate in mission planning and briefings. The GST is not organized, trained, or equipped to provide area support; however, the team is expected to leverage and work in conjunction with other medical assets that are attached to the supported unit when needed (such as a mass casualty event). In certain situations, the GST may combine with another medical asset for the duration of the mission or for a limited basis to create a theater medical capability (for example, co-locating a GST with a medical evacuation element). Upon arrival, GST should determine what other medical capabilities are available at the supported location to ensure a complete medical common operating picture.

5.2. Expeditionary Combat Support (ECS)/Base Operating Support (BOS) Requirements. The GST deploys with limited organic capability and requires ECS/BOS. ECS/BOS requirements include (but are not limited to) billeting, shelter, messing and other consumable materials, power, water, ice, latrines, showers, laundry, waste management, exterior lighting, transportation (to include ambulances and general purpose vehicles), fuels, vehicle maintenance, equipment maintenance, general supplies, contracting, information and communications systems support, mortuary affairs, public affairs, chaplain, linguist, personnel and vehicle decontamination, and security. ECS/BOS may be provided using the host base's capabilities, deployable bare base systems, and contracted civilian support. See **Attachment 2, EXPEDITIONARY COMBAT SUPPORT REQUIREMENTS,** for quantified estimates on required support.

Chapter 6

SECURITY

6.1. Security Roles and Responsibilities. Medical personnel and equipment are non-combatant assets and have protected status under the Geneva Conventions and the broader Law of War. Arming requirements for deploying GST personnel are mission and operating location specific and are provided in reporting instructions and theater guidance. Current threat assessments provided by the combatant commander and local threat conditions established by the joint task force, air expeditionary wing, or air expeditionary group commander dictate local security measures. GST members are responsible for ensuring security measures are in place to protect patients and personnel at the medical site. GST personnel are responsible for following the personal protection measures outlined in DAFI 31-101, area of responsibility security briefings, established force protection requirements, and local guidance.

6.2. Physical Security. Security forces guard medical facilities only if deemed necessary as part of the Integrated Defense Plan. If the threat changes, security forces may appoint an augmented detail to provide force protection and entry control (for example, a mass casualty event). The supported unit provides security for any patient determined to require a security detail or escort. The GST does not have the manning to provide this layer of security.

6.3. Operations Security. GST personnel are responsible for protecting mission-critical information (to include medical or casualty information) in accordance with theater policy and AFI 10-701, *Operations Security (OPSEC)*. Classified information should be transmitted by secure means and protected in accordance with Department of Defense Manual (DODM) 5200.01, Volume 3_DAFMAN 16-1404, Volume 3, *Information Security Program: Protection of Classified Information.* Situation reports, medical surveillance, site locations, and compiled patient data are examples of information that may be classified and require protection.

6.4. Security of Weapons and Ammunition. Weapons and ammunition should be secured in accordance with DAFI 31-101 and local procedures. In austere environments where access to an armory may not be available, GST members are responsible for safe storage of weapons and ammunition until the patient's unit can assume responsibility. GST members should coordinate with the supported unit on local weapons handling guidance. Considerations include rules of engagement on searching and clearing patients of weapons and explosives, who is allowed to carry weapons into the medical treatment area (for example, medical facility, shelter of opportunity), and safety measures for handling the discovery of unauthorized weapons or explosives.

Chapter 7

TRAINING

7.1. Medical Readiness Training Requirements. Personnel assigned to the GST must complete the Comprehensive Medical Readiness Program (CMRP) training requirements specified in DAFI 41-106, *Medical Readiness Program,* the CMRP Category III Training Guide, and MEFPAK responsible agency guidance. Additional training that may be required to meet operational or theater-specific requirements is identified in deployment reporting instructions.

7.2. UTC Formal Training. GST personnel should complete in-residence formal training in accordance with the CMRP Category III Training Guide. UTC team integrity should be maintained to the maximum extent possible. Training schedules and additional course information are available from the ACC/SG Exercise and Training Branch (**accsg.sgxt.utcformaltraining@us.af.mil**).

7.2.1. The Ground Surgical Team Training – Austere Phase 1 course provides foundational training on how to function as a small surgical team. The course focuses on allowance standard familiarization, individual and cross-functional team roles and responsibilities, clinical practice guidelines, blood management, and scenario-based training. Team members receive specialized training in battlefield blood acquisition and management processes and walking blood bank procedures to supplement blood product inventory in emergency situations. The course is held at the United States School of Aerospace Medicine (USAFSAM), Wright-Patterson Air Force Base, Ohio.

7.2.2. The Ground Surgical Team Training – Austere Phase 2 course runs concurrently with the EMEDS formal course and includes an extra day for proficiency testing. GST members will attend various segments of the EMEDS course (such as shelter setup and pack out) as well as segments tailored to the GST austere mission. The Phase 2 course culminates in a capstone field exercise that provides realistic field operational training tied to the team's mission essential tasks and deployment scenarios. Attendees receive credit for EMEDS and GST training. The course is held at the Camp Bullis Medical Readiness Training site, Joint Base San Antonio, Texas.

7.3. Mission Essential Task Lists (METLs). GST personnel should be proficient in the EMEDS Core METLs and their UTC-specific METLs. METLs are available on the ACC/SG MEFPAK Playbook (**https://usaf.dps.mil/sites/12173/SitePages/ACC-SG-Ground-Medical-UTC-Management.aspx**) or from the ACC/SG Exercise and Training Branch.

7.4. Pallet Buildup Training. Pallet buildup training is required for the health services administrator (041A) and surgical technologist (4N1) positions. Training is conducted at the base level. Personnel attend refresher training as needed.

7.5. Hazardous Cargo Training. Hazardous cargo training is required for the health services administrator (041A) and surgical technologist (4N1) positions. Training is conducted at the base level. Personnel attend refresher training as needed.

7.6. Weapons Training. Weapons training requirements are outlined in DAFI 31-131, *Combat Arms Training and Maintenance (CATM) Program,* and DAFI 41-106. Theater combatant commands may impose additional or more stringent requirements, which are generally specified in the operation's execution order or reporting instructions.

7.7. Medical Logistics Training Assistance. The Air Force Medical Logistics Operations Center (AFMLOC) can provide just-in-time training for the health services administrator (041A) and other GST personnel with medical logistics responsibilities at the deployed location. This pre-deployment training covers topics such as setting up a Department of Defense Activity Address Code (DODAAC), supply chain processes, and how to coordinate medical equipment maintenance and repair. Contact the AFMLOC by email at **usaf.detrick.afmra.mbx.sg4m-readiness-afmloc@health.mil** or by phone at DSN 312-343-2883/4172/1094.

7.8. Communications and Information Systems Training. GST members should be trained in the proper use of the communications and information systems included in their equipment package. See the CMRP Category III Training Guide for more information on the training platform, attendance, and currency requirements.

Chapter 8

LOGISTICS

8.1. Medical Logistics Support. The GST may deploy to environments with limited medical logistics support. Planners should ensure the team deploys with adequate medical supplies and equipment to support independent operation requirements. Austere missions may not allow or require establishment of a resupply mechanism. When resupply is required, medical planners, in coordination with medical logistics, coordinate resupply through a variety of mechanisms such as a host medical facility, in-country embassy, military assistance group, or main operating base. When deployed in support of a major regional conflict, the conventional health support system provides medical logistics support. Upon notification of activation, the UTC team chief or logistics lead should coordinate with the AFMLOC and the supported JTF/SG or AFFOR/SG for guidance on the appropriate theater supply chain and points of contact. See Department of the Air Force Tactics, Techniques, and Procedures (DAFTTP) 3-42.8, *Expeditionary Medical Logistics (EML) System,* for more information on medical logistics support processes.

8.2. Blood Support. The Armed Services Blood Program provides blood products for GST missions. Requests for blood supplies are routed through the Joint Task Force Surgeon's Area Joint Blood Program Office representative in accordance with Air Force Handbook (AFH) 44-152, *Armed Services Blood Program – Joint Blood Program Handbook.* The GST primarily relies on stored blood products for casualty resuscitations. They can perform limited emergency whole blood collection and transfusion to augment their blood product inventory in emergency situations.

8.3. Supplies and Equipment. The ground surgical equipment set (FFGS1/AS 938Q) consists of portable field packs designed to support up to 10 damage control surgeries. It is designed for short-duration, single missions. The equipment set is configured in three increments that offer some degree of built-in resupply. Increment 1 provides the initial durable and non-durable items to handle 1-3 surgical cases. It consists of 6 bags (A1, A2, B1, B2, C, E). Increment 2 provides supplemental non-durable items to handle an additional 3 surgical cases (patients 4-6). It consists of 3 bags (G1, G2, G3). Increment 3 is packed identically to increment 1 except the durable items are removed, and non-durable items are added to handle 4 surgical cases (patients 7-10). It consists of 6 bags (H1, H2, H3, H4, H5, H6). Resupply or reconstitution of supplies can be done by line item, sub-package, or increment. For example, if the durable items are still functional, the team can alternate increment 2 and increment 3 packages to replenish supplies. The health services administrator is typically responsible for resupply functions. Team members should contact their local medical logistics flight for assistance with reviewing the allowance standard.

8.4. Medical Equipment Maintenance and Repair. The GST relies on external support for routine maintenance and repair of medical equipment while deployed. Maintenance and repair support agreements are established by the AFFOR/SG in conjunction with the AFMLOC before deployment.

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Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

References

DODI 2310.08, *Medical Program Support for Detainee Operations,* 5 September 2019

DODI 6490.03, *Deployment Health*, 19 June 2019

DODM 5200.01V3_DAFMAN 16-1404V3, *Information Security Program: Protection of Classified Information,* 12 April 2022

DAFI 24-602V2, *Cargo Movement,* 20 December 2023

DAFI 31-101, *Base Defense Operations,* 10 September 2024

DAFI 31-131, *Combat Arms Training and Maintenance (CATM) Program,* 1 April 2025

DAFI 41-106, *Medical Readiness Program,* 29 July 2020

DAFI 41-230, *Medical Logistics Support,* 24 July 2025

DAFI 48-107V3, *En Route Care Documentation,* 17 December 2020

DAFMAN 17-1301, *Computer Security (COMPUSEC)*, 9 December 2024

DAFMAN 21-201, *Munitions Management,* 24 October 2024

DAFTTP 3-42.8, *Expeditionary Medical Logistics (EML) System,* 15 June 2021

AFI 10-204, *Air Force Service Exercise Program and Support to Joint and National Exercise Program,* 6 September 2023

AFI 10-403, *Deployment Planning and Execution,* 17 April 2020

AFI 10-701, *Operations Security (OPSEC),* 24 July 2019

AFI 10-1302, *Air Force Lessons Learned Program,* 30 July 2019

AFI 17-130, *Cybersecurity Program Management,* 13 February 2020

AFI 33-322, *Records Management and Information Governance Program,* 23 March 2020

AFI 33-332, *Air Force Privacy and Civil Liberties Program,* 10 March 2020

AFH 44-152, *Armed Services Blood Program – Joint Blood Program Handbook,* 1 December 2011

AFMAN 10-206, *Operational Reporting (OPREP),* 18 June 2018

AFMAN 24-604, *Preparing Hazardous Materials for Military Air Shipments,* 6 June 2025

AFMAN 41-210, *Patient Administration Support,* 26 September 2025

AFPAM 10-219V5, *Bare Base Conceptual Planning,* 30 March 2012

AFPAM 10-219V6, *Planning and Design of Expeditionary Air Bases,* 1 February 2006

AFPAM 10-1403, *Air Mobility Planning Factors,* 24 October 2018

AFTTP 3-42.71, *Expeditionary Medical Support (EMEDS) and Air Force Theater Hospital (AFTH)*, 17 September 2024

Defense Transportation Regulation (DTR) 4500.9-R-Part III, *Mobility,* June 2016

United States Air Force, *War and Mobilization Plan, Volume 1 (WMP-1), Enclosure F, Medical Supplement*, November 2020

Adopted Forms

DD 1380, *Tactical Combat Casualty Care (TCCC) Card*

DD 3019, *Resuscitation Record*

SF 600, *Chronological Record of Medical Care*

DAF 847, *Recommendation for Change of Product*

AF 3899, *Patient Movement Record*

Abbreviations and Acronyms

ACC—Air Combat Command

AFFOR—Air Force Forces

AF—Air Force (form)

AFH—Air Force Handbook

AFI—Air Force Instruction

AFMAN—Air Force Manual

AFMEDCOM—Air Force Medical Command

AFMLOC—Air Force Medical Logistics Operations Center

AFOMIS—Air Force Operational Medicine Information System

AFPAM—Air Force Pamphlet

AFSC—Air Force Specialty Code

AFSOC—Air Force Special Operations Command

AFTH—Air Force Theater Hospital

AFTTP—Air Force Tactics, Techniques, and Procedures

AMC—Air Mobility Command

BOS—Base Operating Support

C2—Command and Control

CASEVAC—Casualty Evacuation

CMRP—Comprehensive Medical Readiness Program

COMAFFOR—Commander, Air Force Forces

DAF—Department of the Air Force (form)

DAFI—Department of the Air Force Instruction

DAFMAN—Department of the Air Force Manual

DAFTTP—Department of the Air Force Tactics, Techniques, and Procedures

DD—Department of Defense (form)

DOD—Department of Defense

DODAAC—Department of Defense Activity Address Code

DODI—Department of Defense Instruction

DODM—Department of Defense Manual

DTR—Defense Transportation Regulation

ECU—Environmental Control Unit

ECS—Expeditionary Combat Support

EMEDS—Expeditionary Medical Support

GST—Ground Surgical Team

HIPAA—Health Insurance Portability and Accountability Act

JTF—Joint Task Force

MEFPAK—Manpower and Equipment Force Packaging

METL—Mission Essential Task List

NIPRNET—Non-classified Internet Protocol Router Network

OPR—Office of Primary Responsibility

SF—Standard Form

SG—Surgeon General, Surgeon

SIPRNET—SECRET Internet Protocol Router Network

TMIP—Theater Medical Information Program

USAFSAM—United States School of Aerospace Medicine

UTC—Unit Type Code

VOIP—Voice Over Internet Protocol

VOSIP—Voice Over Secure Internet Protocol

WMP—War and Mobilization Plan

Office Symbols

ACC/SG—Air Combat Command Surgeon

ACC/SGX—Air Combat Command Expeditionary Operations Division

AFMEDCOM/A4/10—Air Force Medical Command Expeditionary Operations Division

AFFOR/SG—Air Force Forces Surgeon

JTF/SG—Joint Task Force Surgeon

Attachment 2

EXPEDITIONARY COMBAT SUPPORT REQUIREMENTS

A2.1. GST Expeditionary Combat Support Requirements.

Table A2.1. GST Expeditionary Combat Support Requirements.

GST Expeditionary Combat Support Requirements	
Calculations are in accordance with AFPAM 10-219, Vols 5 & 6, where applicable unless otherwise specified.	
	FFGST (Personnel) **FFGS1 (Equipment)**
MOVEMENT REQUIREMENTS **Calculations are in accordance with AFPAM 10-1403 and DTR 4500.9-R-Part III**	
Pallets (#)	1
C-130 (# aircraft)	1
C-17 (# aircraft)	1
C-5 (# aircraft)	1
M871 (# flatbed semitrailers)	1
M872 (# flatbed semitrailers)	1
SITE PREPARATION	
Square Footage (slight grade required)	
Tents (#)	
ECUs (#)	
Note: The GST operates from a shelter of opportunity when not attached to an EMEDS or equivalent deployed medical facility.	
BASIC EXPEDITIONARY AIRFIELD RESOURCES (BEAR) REQUIREMENTS	
Latrine/Showers (# staff)	6
Billeting (# staff)	6
# Officers	5
# Enlisted	1
Meals (# meals/day) (= # staff x 3 meals/day)	18
Laundry (lbs/week) (= 32 lbs/person/week)	192
Ice (lbs/day) (= 4.4 lbs/person/day)	26.4
Potable Water (gal/day) (= # staff x 10 gal/day)	60
Power (kW)	6
Note: The GST allowance standard includes a 1.5 kW generator intended for initial operating capability and emergency backup power only.	
MEDICAL/BIOHAZARDOUS WASTE MANAGEMENT	

GST Expeditionary Combat Support Requirements	
Calculations are in accordance with AFPAM 10-219, Vols 5 & 6, where applicable unless otherwise specified.	
	FFGST (Personnel) FFGS1 (Equipment)
Liquid (gal/day) (= 0.7 x potable water rate)	42
Solid (lbs/day) (= 4 lbs x #staff)	24
LOGISTICS REQUIREMENTS	
Petroleum, Oils, Lubricants	
Diesel Fuel (gal/day) (= 8.33 gal/hr x 24 hrs)	
Unleaded Fuel (gal/day) (10kW generator)	
Vehicles	
Vehicle Maintenance Support	
Vehicle Requirements	
Material Handling Equipment	
COMMUNICATIONS AND INFORMATION SYSTEMS REQUIREMENTS	
Organic Equipment	
Satellite Phone (# equipment)	1
Laptop (# equipment)	1
Printers (# equipment)	
Server Suite (# equipment)	
Base Communications Support Requirements	
Phone/VOIP Access (#)	2
Land Mobile Radio Access (#)	2
SIPRNET/VOSIP Access	Required
NIPRNET Access	1 ethernet drop required
Operating System/Office Suite	DOD SDC
RAM/Hard Drive	Enterprise hardware standard
Clinical Applications	TMIP
Required Port Number/Protocol Access (TCP/UDP)	21/TCP; 443/TCP; 8080/TCP
CHAPLAINCY SERVICE SUPPORT	
	Required
SECURITY FORCES SUPPORT	
	Required if not co-located on Air Base